CHAPTER

07

Psychological Disorders and Treatment Techniques

Many people feel sad, worried or stressed from time to time; that is normal. A mental-health concern becomes a **psychological disorder** when signs and symptoms last, cause frequent distress and interfere with the ability to function at school, at work or in relationships. This chapter explains how disorders are identified, what causes them according to the major perspectives, the main types (mood, anxiety and personality disorders), and how they are treated. Most disorders can be managed with psychological treatment, medication or both.

LESSONS IN THIS CHAPTER

7.1 The Nature of Psychological Disorders

7.2 Causes of Psychological Disorders: The Perspectives

7.3 Mood Disorders

7.4 Anxiety Disorders

7.5 Personality Disorders

7.6 Treatment Techniques

Freshman A+ · General Psychology (Psyc 1011)

CHAPTER 7 · LESSON 7.1

The Nature of Psychological Disorders



AFTER THIS LESSON YOU CAN

- Explain the three criteria for identifying a psychological disorder
- Explain why no single criterion is enough
- Apply the criteria to cases

7.1.1 Three criteria

D PSYCHOLOGICAL DISORDER

A condition characterized by **abnormal thoughts, feelings and behaviours**. **Psychopathology** is the study of psychological disorders: their symptoms, **etiology** (causes) and treatment.

F THE THREE CRITERIA

Criterion	Question to ask	Meaning
1. Abnormality (deviance)	Does the behaviour **deviate from the norm** of the "typical" person and the society's standards?	culturally unacceptable behaviour that violates social norms; the **context** must always be considered
2. Maladaptiveness	Does it **seriously disrupt** social, academic, occupational or daily life?	inability to function day to day
3. Personal distress	Does it cause the person **anxiety, depression, hopelessness** or self-defeating thoughts?	the **subjective** feeling of suffering

★ NO CRITERION IS ENOUGH ALONE

- **Abnormality alone is not sufficient:** a behaviour that is unusual may be normal in its context (crying loudly at a funeral, a religious practice, a genius's unusual talent).
- **Distress alone is not sufficient:** some people feel distressed by normal events, and some people with serious disorders feel no distress.
- So a behaviour that is **abnormal, maladaptive or personally distressing** *might* indicate a disorder; professionals consider all three together, plus context.

◉ APPLYING THE CRITERIA (MODULE CASES)

- A young woman who did very well in high school struggles at university, feels lonely, and becomes increasingly depressed and withdrawn → **personal distress** and growing **maladaptiveness** (studies affected).
- A middle-aged businessman, fed up with his stressful job, packs a bag and goes to live alone in the mountains → **unusual (deviant)**, but whether it is a disorder depends on context, function and distress.

Exam Practice

5 QUESTIONS · LESSON 7.1

1 Wolaita Sodo University · Final 2014

The presence of abnormal behaviour alone is sufficient for diagnosing a psychological problem.

True / False

2 Arba Minch University · Final 2016

A subjective feeling of anxiety, used as a criterion of psychological disorder, refers to:

- **A)** Abnormality
- **B)** Maladaptiveness
- **C)** Dangerousness
- **D)** Personal distress

3 Jigjiga University · Final (Set A)

The criterion that defines psychological disorders by the inability to function on a daily basis is:

- **A)** Personal distress
- **B)** Cultural deviance
- **C)** Maladaptiveness
- **D)** All of these

4 Arba Minch University · Final 2015

A behaviour is said to be "deviant" (abnormal) if:

- **A)** the person's actions are inappropriate for the expected norm
- **B)** it causes social or personal difficulties
- **C)** the person feels unpleasant emotions
- **D)** it is dangerous to others

5 Model question (module)

Psychopathology is best defined as:

- **A)** the study of normal personality
- **B)** the study of psychological disorders, including their symptoms, causes and treatment
- **C)** the use of drugs to treat illness
- **D)** the study of the brain only

Answers & Explanations

1. False

The context must be considered, and maladaptiveness and personal distress also matter.

2. D

Personal distress

Personal distress is the person's own unpleasant emotional state.

3. C

Maladaptiveness

Maladaptive behaviour disrupts social, academic or occupational functioning.

4. A

the person's actions are inappropriate for the expected norm

Deviance means departing from social norms. B describes maladaptiveness; C describes personal distress.

5. B

the study of psychological disorders, including their symptoms, causes and treatment

Psychopathology covers symptoms, etiology (causes) and treatment of disorders.

CHAPTER 7 · LESSON 7.2

Causes of Psychological Disorders: The Perspectives

AFTER THIS LESSON YOU CAN

- Explain the biological perspective, including neurotransmitters
- Explain the psychoanalytic, learning and cognitive perspectives
- Identify the perspective behind any explanation

7.2.1 Five explanations

WHAT CAUSES DISORDERS? IT DEPENDS ON THE PERSPECTIVE

Perspective	Cause of disorder	Example
Biological	abnormal **brain structure**, **neurotransmitters** (brain chemicals), genes	overactivity of the neurotransmitter **dopamine** is linked to the bizarre symptoms of **schizophrenia**
Psychoanalytic	the **ego's failure** to manage the conflict between the **id** (biological urges) and **superego** (moral standards), especially **unresolved childhood conflicts**	unresolved childhood sexual and emotional conflicts produce adult anxiety
Learning (behavioural)	**inadequate or inappropriate learning**: conditioning and observation	a fear of dogs learned after being bitten; aggression learned by imitation
Cognitive	**irrational, faulty, self-defeating thoughts** ("I am worthless", "I will certainly fail") lead to negative emotions and self-destructive behaviour	a student who believes one bad grade means she is stupid becomes depressed
Humanistic	**blocked growth** and **incongruence** between self-concept and reality (chapter 6)	a student forced into a career he does not value

Scientific psychology does **not** explain disorders by evil spirits or supernatural forces.

PERSPECTIVE TRAPS

- "Improper functioning of brain chemicals" is the **biological** view, not the cognitive view.
- "Inadequate learning from the social environment" is the **learning** view, not the cognitive view.
- "Inappropriate learning causes disorders" is **not** the biological view.
- The **psychoanalytic** view stresses unconscious conflict between biological urges and moral standards.

Exam Practice

7 QUESTIONS · LESSON 7.2

1 Wolaita Sodo University · Mid 2014

According to the biological perspective, psychological disorders result from inappropriate learning.

True / False

2 Wolaita Sodo University · Mid 2014

A perspective that analyses brain structure and nervous-system function as the cause of psychological disorders is the:

A) psychoanalytic perspective
B) behaviourist perspective
C) cognitive perspective
D) biological perspective

3 Arba Minch University · Final 2015

For cognitive theorists, psychological disorders are caused by the improper functioning of brain chemicals.

True / False

4 Arba Minch University · Final 2016

Which of the following is NOT a cognitive explanation for psychological disorders?

A) Irrational thinking about some things
B) Self-defeating negative thoughts
C) Inadequate learning from the social environment
D) Faulty interpretation of events

5 Arba Minch University · Final 2016

Which perspective emphasizes unconscious conflicts between biological urges and society's moral standards in the development of abnormal behaviour?

A) Cognitive
B) Learning
C) Biological
D) Psychoanalytic

6 Jigjiga University · Final (Set A)

Which approach claims that psychological disorders arise from irrational thinking patterns?

A) Cognitive
B) Behavioural
C) Humanistic
D) Biological

7 Arba Minch University · Final 2015

According to psychological perspectives, psychological disorders are caused by all of the following EXCEPT:

A) unresolved childhood conflicts
B) inappropriate learning
C) possession by evil spirits
D) irrational thinking processes

Answers & Explanations

1. False

That is the learning perspective. The biological view points to the brain, neurotransmitters and genes.

2. D

biological perspective

Brain structure and chemistry are biological causes.

3. False

Brain chemicals belong to the biological perspective. Cognitive theorists blame irrational thinking.

4. C

Inadequate learning from the social environment

Inadequate learning is the learning (behavioural) explanation.

5. D

Psychoanalytic

Freud saw disorders as the ego's failure to manage id–superego conflict.

6. A

Cognitive

The cognitive view links faulty, self-defeating thoughts to disorders.

7. C

possession by evil spirits

A, B and D are psychoanalytic, learning and cognitive explanations; scientific psychology does not explain disorders by spirits.

CHAPTER 7 · LESSON 7.3

Mood Disorders

AFTER THIS LESSON YOU CAN

- Define mood disorders
- Describe major depression, dysthymia, bipolar disorder and cyclothymia
- Distinguish the disorders by severity and pattern

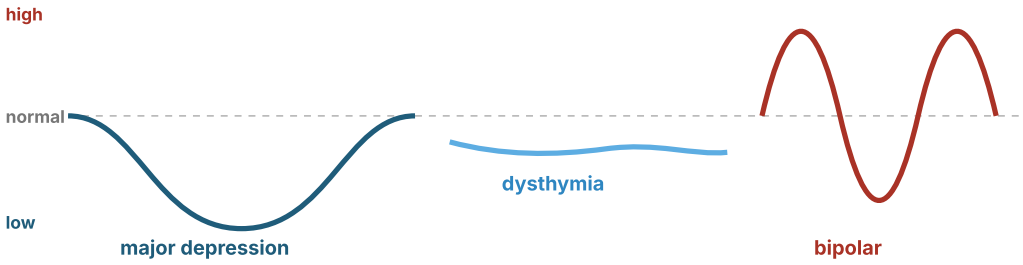
7.3.1 What mood disorders are

D MOOD DISORDERS

Disorders whose main feature is a **serious disturbance of mood**, either abnormally **low** (depression) or abnormally **high** (mania), that is out of proportion to the person's situation and disrupts life.

F THE FOUR MOOD DISORDERS

Disorder	Pattern	Key features
Major depression (clinical depression)	severe **lows**	depressed mood; loss of interest in activities once enjoyed; weight and sleep disturbance; loss of energy; difficulty concentrating; feelings of **hopelessness**; thoughts of suicide
Dysthymia (dysthymic disorder)	**milder but more persistent** depression	similar symptoms to a lesser degree; **steady**, with **less swing** in mood
Bipolar disorder (formerly manic-depression)	**extreme highs (mania)** alternating with **extreme lows**	type I: extreme (manic) episodes; type II: moderate (hypomanic) episodes
Cyclothymia	**milder** form of bipolar disorder	mild ups and downs
Pairs to remember: dysthymia is to major depression as cyclothymia is to bipolar disorder (the milder, longer-lasting form).		



Mood patterns: a deep low, a mild persistent low, and extreme swings

MOOD TRAPS

- **Suspiciousness** is not a symptom of major depression (it belongs to paranoid patterns).
- The mood disorder with the **least swing** is **dysthymia**.
- Feelings of both happiness (mania) and sadness alternating = **bipolar** disorder.
- **Dysthymic** disorder is a mood disorder, not a personality disorder.

Exam Practice

6 QUESTIONS · LESSON 7.3

1 Wolkite University · Final 2015

Mood disorders are syndromes whose main feature is a disturbance in mood, which can be abnormally low or abnormally high.

True / False

2 Wolaita Sodo University · Final 2014

For the past six months, Julie has felt exhausted and hopeless and has been unable to enjoy her normal activities at work. The most likely diagnosis is:

A) dissociative disorder
B) schizotypal personality disorder
C) seasonal affective disorder
D) depression

3 Arba Minch University · Final 2015

Which of the following is NOT a characteristic of major depression?

A) Feelings of hopelessness
B) Suspiciousness
C) Lack of concentration
D) Thoughts of suicide

4 Arba Minch University · Final 2016

Which mood disorder is characterized by less swing in mood?

A) Cyclothymia
B) Dysthymia
C) Bipolar disorder
D) Major depression

5 Jigjiga University · Final (Set A)

A mood disorder characterized by alternating feelings of extreme happiness and sadness is:

A) Dysthymia
B) Cyclothymia
C) Major depression
D) Bipolar disorder

6 Model question (module)

Cyclothymia is best described as:

A) a severe form of major depression
B) a milder form of bipolar disorder
C) an anxiety disorder
D) a personality disorder

Answers & Explanations

1. True

Depression is the low end; mania is the high end.

2. D

depression

Persistent hopelessness, fatigue and loss of interest are core symptoms of depression.

3. B

Suspiciousness

Suspiciousness is not listed among depression's symptoms; it relates to paranoid patterns.

4. B

Dysthymia

Dysthymia is steadier: a milder, persistent low mood without extreme swings.

5. D

Bipolar disorder

Bipolar disorder cycles between mania (extreme highs) and depression (extreme lows). Cyclothymia is a milder version.

6. B

a milder form of bipolar disorder

Cyclothymia involves milder highs and lows than bipolar disorder.

CHAPTER 7 · LESSON 7.4

Anxiety Disorders

AFTER THIS LESSON YOU CAN

- Distinguish normal anxiety from an anxiety disorder
- Describe panic disorder, agoraphobia, phobias, OCD, PTSD and GAD
- Identify the anxiety disorder in a case

7.4.1 Normal anxiety vs anxiety disorder

Anxiety is a normal reaction to stress and can be helpful: it alerts us to danger and helps us prepare (some nervousness before an exam helps concentration). It becomes a **disorder** when the fear or anxiety is **out of proportion** to the situation (or inappropriate for age) and **hinders normal functioning**. Anxiety disorders are the **most common** mental disorders (affecting nearly 30 % of adults at some point) and are **treatable**.

TYPES OF ANXIETY DISORDER

Disorder	Key feature	Typical case
Panic disorder	repeated **panic attacks**: sudden intense terror with **pounding heart**, trembling, shortness of breath, chest pain, **dizziness**, a sense of dying; often mistaken for a heart attack	a student suddenly becomes dizzy, sweats and faints during an exam
Agoraphobia	fear and avoidance of places where **escape or help might not be available** ("fear of the marketplace"): markets, shopping centres, public places	Mr. Gemechu avoids public places because he worries he cannot escape
Specific (simple) phobia	intense fear of a **specific object or situation**	Halimo's extreme fear of rats; fear of heights or snakes
Social phobia	intense fear of **social situations**: being judged or embarrassed when interacting with others	
Obsessive–compulsive disorder (OCD)	**obsessions** (uncontrollable thoughts) and **compulsions** (repeated behaviours to reduce them): checking, counting, cleaning, washing	Kidusan checks the light switches, taps, stove and doors at least three times before bed
Post-traumatic stress disorder (PTSD)	after a **traumatic event** (war, disaster, serious accident, abuse): **flashbacks**, **nightmares**, avoidance, heightened anxiety	a student from a war-affected area has nightmares and flashbacks; an earthquake survivor feels as if it is happening now
Generalized anxiety disorder (GAD)	extreme anxiety about **nearly every part of life**, not tied to one object, place or event	

TELLING ANXIETY DISORDERS APART

Ask three questions:

1. Is there a past **traumatic event**, with flashbacks or nightmares? → **PTSD**.
2. Is the fear tied to **something specific**? A specific object → **specific phobia**; social situations → **social phobia**; places where escape is hard → **agoraphobia**.
3. Is it **sudden attacks** of terror with physical symptoms? → **panic disorder**. **Repeated thoughts and rituals**? → **OCD**. **Worry about everything**? → **GAD**.

ANXIETY TRAPS

- **Specific** phobia is fear of a particular thing; fear of **interacting with others** is **social** phobia.
- **Agoraphobia** is about places where **escape** is difficult, not about one object.
- GAD involves anxiety about **every aspect of life**.

Exam Practice

10 QUESTIONS · LESSON 7.4

1 Arba Minch University · Final 2016

A freshman from northern Ethiopia has nightmares and flashbacks of wartime conditions. Which disorder is the student most likely experiencing?

A) Agoraphobia
B) Post-traumatic stress disorder
C) Obsessive–compulsive disorder
D) Personality disorder

2 Arba Minch University · Final 2015

Hazal survived the earthquake in Turkey, but she now reports that the event disturbs her as if it were happening now. Hazal's disorder is:

A) Generalized anxiety
B) Panic attacks
C) Post-traumatic stress
D) Obsessive–compulsive

3 Arba Minch University · Final 2016

Mr. Gemechu fears going to public places and avoids them because he worries about being unable to escape. He is likely suffering from:

A) Panic disorder
B) Agoraphobia
C) Social phobia
D) Post-traumatic stress disorder

4 Jigjiga University · Final (Set A)

Halimo has an extreme fear of rats. She is most likely experiencing:

A) major depression
B) agoraphobia
C) specific phobia
D) generalized anxiety disorder

5 Arba Minch University · Final 2016

Specific phobia involves fear of interacting with others.

True / False

6 Wolaita Sodo University · Final 2014

Kidusan checks the light switches, water taps and stove, and locks the doors at least three times before going to bed. This describes:

A) Panic disorder
B) Phobia
C) Obsessive–compulsive disorder
D) Post-traumatic stress disorder

7 Jigjiga University · Final (Set A)

Obsessive–compulsive disorder is characterized by:

A) repetitive thoughts and behaviours
B) irrational fear of specific objects
C) a deep state of sadness
D) an excessively euphoric mood

8 Jigjiga University · Final (Set A)

A person suffers sudden feelings of terror with a racing heartbeat, sweating and a sense of dying. These symptoms characterize:

A) generalized anxiety disorder
B) agoraphobia
C) post-traumatic stress disorder
D) panic disorder

9 Arba Minch University · Final 2015

A student who faints and becomes dizzy in class during a final exam is most probably experiencing:

A) panic disorder
B) specific phobia
C) generalized anxiety disorder
D) post-traumatic stress disorder

10 Arba Minch University · Final 2015

Generalized anxiety disorder is characterized by excessive anxiety about:

A) public situations
B) specific things
C) poor functioning
D) every aspect of life

Answers & Explanations

1. B
Post-traumatic stress disorder
Flashbacks and nightmares after a life-threatening event are the hallmark of PTSD.

2. C
Post-traumatic stress
Re-experiencing a trauma (flashbacks) is PTSD.

3. B
Agoraphobia
Fear of places where escape or help may be unavailable is agoraphobia.

4. C
specific phobia
Intense fear of one specific object is a specific phobia.

5. False
Fear of social interaction is social phobia; specific phobia is fear of a particular object or situation.

6. C
Obsessive–compulsive disorder
Repeated checking (compulsions) driven by uncontrollable worry (obsessions) is OCD.

7. A
repetitive thoughts and behaviours
Obsessions are repetitive thoughts; compulsions are repetitive behaviours.

8. D
panic disorder
Sudden, intense attacks of fear with physical symptoms are panic attacks.

9. A

panic disorder

Dizziness and fainting in a sudden wave of intense fear match a panic attack.

10. D

every aspect of life

GAD is not tied to one object, place or event; it spreads across nearly all of life.

CHAPTER 7 · LESSON 7.5

Personality Disorders

AFTER THIS LESSON YOU CAN

- Define personality disorder
- Describe the nine personality disorders
- Identify the disorder from a description

7.5.1 What a personality disorder is

D PERSONALITY DISORDER

A **rigid, unhealthy, long-standing** pattern of thinking, feeling and behaving that differs markedly from the person's culture, makes it hard to perceive and relate to situations and people, and causes distress or negative consequences. The key word is **inflexible**.

F THE NINE PERSONALITY DISORDERS

Disorder	Pattern	Clue words
Paranoid	**distrust and suspiciousness** of others	"they are plotting against me", with no evidence
Schizoid	**detachment** from social relationships; restricted emotions; little interest in others	prefers to be alone; emotionally cold
Schizotypal	discomfort in close relationships plus **eccentric** thoughts and behaviour	odd beliefs, strange behaviour
Antisocial	**disregard for the rights of others**; violating them without **empathy** or guilt	lying, cheating, cruelty without remorse
Borderline	**instability** in relationships and emotions, switching rapidly between clinging affection and anger	"I love you / I hate you"
Histrionic	**excessive emotionality** and **attention seeking**	dramatic, must be the centre of attention
Narcissistic	**grandiosity**, exaggerated self-worth, need for **admiration**	"I am the only handsome and intelligent one"
Avoidant	feelings of **social inadequacy**, low self-esteem, **hypersensitivity to criticism**	avoids people for fear of rejection
Obsessive–compulsive (personality)	preoccupation with **cleanliness, perfection and control**	rigid perfectionism (different from OCD, the anxiety disorder)

◉ SAME STUDENT, WHICH DISORDER?

Sara has always been cautious and distrustful; she constantly suspects that friends and co-workers talk behind her back and plot against her, without evidence → **paranoid**. John, a 4th-year student, believes he is the only handsome and intelligent student at the university → **narcissistic**. A classmate who must always be the centre of attention and reacts dramatically → **histrionic**. A student who avoids group work because he expects criticism → **avoidant**.

PERSONALITY-DISORDER TRAPS

- **Narcissistic** people show **inflated** self-worth, not poor self-esteem. (Low self-esteem and sensitivity to criticism point to **avoidant**.)
- A **sense of inadequacy** is a feature of **avoidant**, not histrionic, personality disorder.
- **Schizoid** = little interest in social relationships (true).
- **Dysthymic** disorder is a **mood** disorder, not a personality disorder.

Exam Practice

9 QUESTIONS · LESSON 7.5

1 Arba Minch University · Final 2015

A personality disorder is characterized by:

A) a general feeling of desperation and inactivity
B) an inflexible pattern of behaviour and thinking
C) a serious level of fear
D) intense mood swings

2 Arba Minch University · Final 2016

Sara has always been cautious and distrustful. She constantly suspects that her friends and co-workers talk behind her back and plot against her, even when there is no evidence. Her pattern fits which personality disorder?

A) Schizoid
B) Narcissistic
C) Histrionic
D) Paranoid

3 Arba Minch University · Final 2015

John, a 4th-year student, believes he is the only handsome and intelligent student in the university. Which personality disorder does this suggest?

A) Narcissistic
B) Paranoid
C) Borderline
D) Antisocial

4 Arba Minch University · Final 2016

Which personality disorder is characterized by excessive emotional behaviour and a constant need for attention?

A) Histrionic
B) Schizotypal
C) Obsessive–compulsive
D) Paranoid

5 Arba Minch University · Final 2015

Schizoid personality disorder is characterized by little interest in social relationships.

True / False

6 Arba Minch University · Final 2015

People with narcissistic personality disorder have poor self-esteem.

True / False

7 Arba Minch University · Final 2016

A sense of inadequacy is a symptom of histrionic personality disorder.

True / False

8 Arba Minch University · Final 2015

Which one of the following is NOT a type of personality disorder?

A) Obsessive–compulsive
B) Avoidant
C) Antisocial
D) Dysthymic

9 Model question (module)

A man repeatedly lies, cheats and hurts others for personal gain and shows no guilt or empathy. This pattern fits which personality disorder?

A) Avoidant
B) Antisocial
C) Schizoid
D) Borderline

Answers & Explanations

1. B

an inflexible pattern of behaviour and thinking

Personality disorders are rigid, long-standing patterns. A describes depression; C anxiety; D bipolar disorder.

2. D

Paranoid

Pervasive distrust and suspicion define paranoid personality disorder.

3. A

Narcissistic

Grandiosity and an exaggerated sense of self-worth are narcissistic features.

4. A

Histrionic

Histrionic = dramatic emotionality and attention seeking.

5. True

Schizoid = detachment from relationships and restricted emotions.

6. False

They show an inflated, grandiose sense of self-worth and need admiration.

7. False

Feelings of inadequacy and hypersensitivity to criticism belong to avoidant personality disorder.

8. D

Dysthymic

Dysthymia is a mood disorder.

9. B

Antisocial

Disregard for others' rights and lack of empathy define antisocial personality disorder.

CHAPTER 7 · LESSON 7.6

Treatment Techniques



AFTER THIS LESSON YOU CAN

- State the four core components of psychotherapy and the qualities of a good therapist
- Compare cognitive, behavioural and psychodynamic approaches
- Describe individual, group, couple/family and combined modalities
- Match treatments to disorders

7.6.1 Psychotherapy

Treatment can include **medication**, **talk therapy (psychotherapy)**, or both, and can last one session or many years. Whatever the approach, psychotherapy shares four core components:

1. a **positive, healthy relationship** between the client and a trained therapist;
2. **recognizable** mental-health issues (diagnosable or not);
3. **agreement** on the basic goals of treatment;
4. **working together as a team** to reach them.

A good therapist shows **empathy** (understanding the client's feelings, thoughts and behaviour), is **non-judgemental**, is experienced and well trained, and keeps up with research. The goal is to reduce symptoms, gain **insight** into why they happen, and prevent them from returning.

7.6.2 Treatment approaches

THE THREE MAIN APPROACHES

Approach	Sees the problem as...	Therapy focuses on...	Works especially well for
Cognitive	**irrational or faulty thinking**	changing distorted thoughts and beliefs into rational ones	**depression**
Behavioural	**learned behaviours** maintained by reinforcement	**teaching appropriate ways of acting**; unlearning maladaptive behaviour (e.g. systematic desensitization)	**phobias**
Psychodynamic (dynamic)	**unconscious** issues from **early childhood**	gaining insight into hidden conflicts (Freud's methods: free association, dream analysis)	
Most professionals today are **eclectic** ("**integrationists**"): they combine approaches to fit the client.			

7.6.3 Treatment modalities

Modality	Who takes part	Strength
Individual therapy	one client and a therapist	the **most common** form

Modality	Who takes part	Strength
Group therapy	3–20 people with similar problems, with 1–2 therapists	belonging, feeling understood, hope, support; the **second most used** form
Couple / family therapy	a couple or family	centred on the **relationship**: communication training, listening, respect, teamwork
Combined	e.g. individual therapy first, then group therapy	a person with depression, social anxiety and low self-esteem works on symptoms individually, then practises social skills in a group

GETTING HELP IS A STRENGTH

Anxiety disorders, depression and most other disorders are **treatable**, and treatment helps most people lead normal, productive lives. Every Ethiopian university has a **counselling (guidance) office** where students can talk confidentially about stress, low mood, exam anxiety or family problems. Seeking help early is wise, not a sign of weakness.

TREATMENT TRAPS

- **Behavioural** therapy teaches appropriate behaviour; **creating rational thinking** is cognitive therapy; **understanding early experiences** is psychodynamic.
- The best-supported approach for **depression** is **cognitive**; for **phobias**, **behavioural**.
- The modality centred on **relationships** is **family (couple) therapy**.

Exam Practice

7 QUESTIONS · LESSON 7.6

1 Arba Minch University · Final 2015

The major focus of behavioural therapy is:

A) creating rational thinking processes
B) understanding early experiences
C) teaching appropriate ways of acting
D) restructuring bodily functioning

2 Jigjiga University · Final (Set A)

Which approach works best for treating individuals suffering from depression?

A) Cognitive
B) Behavioural
C) Psychoanalysis
D) Humanistic

3 Arba Minch University · Final 2015

Which treatment modality is mostly centred on relationship issues?

A) Group therapy
B) Individual therapy
C) Family therapy
D) Eclectic therapy

4 Model question (module)

Which treatment approach tends to work best for phobias?

A) Cognitive
B) Behavioural
C) Psychodynamic
D) Group therapy only

5 Model question (module)

Which of the following is NOT one of the four core components of psychotherapy?

- **A)** A positive relationship between client and therapist
- **B)** Agreement on the goals of treatment
- **C)** Working together as a team
- **D)** The therapist judging the client's mistakes

6 Model question (module)

A therapist who combines cognitive, behavioural and psychodynamic techniques to suit each client is described as:

- **A)** eclectic (integrationist)
- **B)** psychoanalytic
- **C)** behaviourist
- **D)** biological

7 Model question (module)

After group sessions, a client says, "For the first time I feel understood; others have the same problem and are getting better." This illustrates the main strength of:

- **A)** individual therapy
- **B)** group therapy
- **C)** medication
- **D)** psychodynamic therapy

Answers & Explanations

1. C

teaching appropriate ways of acting

Behaviour therapy treats disorders as learned behaviour and teaches new, adaptive behaviour. A is cognitive; B is psychodynamic.

2. A

Cognitive

The module states that cognitive approaches appear to work better with most types of depression.

3. C

Family therapy

Couple and family therapy focus on the relationship and communication among members.

4. B

Behavioural

Behavioural methods such as systematic desensitization gradually unlearn the fear.

5. D

The therapist judging the client's mistakes

A good therapist is non-judgemental; judging the client destroys the sense of safety.

6. A

eclectic (integrationist)

Most professionals today are eclectic, using different approaches as needed.

7. B

group therapy

Group therapy meets the need to belong, feel understood and have hope.

CHAPTER 7 · QUICK REVISION

Psychological Disorders and Treatment Techniques — at a glance

CHAPTER 7 AT A GLANCE

Topic	Must-know points
Criteria	abnormality (deviance from norms, consider context), maladaptiveness (cannot function), personal distress; no single criterion is enough
Causes	biological (neurotransmitters, e.g. dopamine and schizophrenia); psychoanalytic (ego fails to manage id–superego conflict); learning (inappropriate learning); cognitive (irrational, self-defeating thoughts)
Mood	major depression; dysthymia (milder, persistent, less swing); bipolar (mania + depression); cyclothymia (milder bipolar)
Anxiety	panic (sudden attacks); agoraphobia (no escape); specific phobia (one thing); social phobia (social situations); OCD (obsessions + compulsions); PTSD (trauma, flashbacks); GAD (everything)
Personality	inflexible patterns: paranoid, schizoid, schizotypal, antisocial, borderline, histrionic, narcissistic, avoidant, obsessive–compulsive
Treatment	core: relationship, recognizable issue, agreed goals, teamwork; approaches: cognitive (depression), behavioural (phobias), psychodynamic; eclectic; modalities: individual, group, couple/family, combined

PERSONALITY DISORDERS: "PARANOID SARA SUSPECTS; NARCISSIST JOHN ADMIRES HIMSELF"

Suspicion → **paranoid**. Self-admiration → **narcissistic**. Attention → **histrionic**. Alone → **schizoid**. Odd → **schizotypal**. No empathy → **antisocial**. Unstable → **borderline**. Fear of criticism → **avoidant**. Perfection → **obsessive–compulsive**.